

CLINICAL PRACTICE UPDATES

AGA Clinical Practice Update on the Role of Therapeutic Endoscopy in Inflammatory Bowel Disease: Commentary



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DESCRIPTION:

The purpose of this American Gastroenterological Association (AGA) Institute Clinical Practice Update (CPU) Commentary is to discuss the different scenarios in which advanced therapeutic endoscopic procedures can be used for patients with inflammatory bowel diseases. The CPU will also inform clinical practice on when such procedures should and should not be considered.

METHODS:

This CPU was commissioned and approved by the AGA Institute CPU committee and the AGA Governing Board to provide timely guidance on a topic of high clinical importance to the AGA membership and underwent internal peer review by the CPU committee and external peer review through standard procedures of Clinical Gastroenterology and Hepatology. This communication incorporates important and recently published studies in the field, reflecting the experiences of the authors.

Keywords: Crohn's Disease; Ulcerative Colitis; Stricture; Neoplasia; Endoscopy.

Introduction

Despite advances in medical management, patients with inflammatory bowel disease (IBD) experience complications such as stricturing and penetrating disease, and colitis-associated neoplasia (CAN). Advanced therapeutic endoscopic interventions offer patients an opportunity to delay or even prevent surgery in some instances. We hereby cover the different scenarios in which advanced therapeutic procedures can be considered for patients with IBD.

Preprocedure Preparation

Interventions in IBD can be performed through various endoscopic approaches. Almost all interventional IBD procedures can be performed electively, and rarely urgent Adequate bowel preparation is critical anticoagulants, antiplatelet agents (except low-dose aspirin), systemic corticosteroids, and glucagon-like peptide-1 receptor agonists should be held prior to elective procedures per current society guidances.¹

Endoscopic Management of Strictures

Strictures, described as luminal narrowing's develop because of chronic inflammation, fibrosis, and smooth

muscle hyperplasia. Population-based studies show that 5%–28% of patients with Crohn's disease (CD) have stricturing disease at presentation, and more than 50% of patients with CD develop stricturing complications within the first 10 years after diagnosis.²

Strictures can be primary or secondary/anastomotic, benign or malignant, inflammatory or fibrostenotic, mild or severe, short (cutoff of 4–5 cm) or long, and simple or complex (the latter with fistula or abscesses), and may be single or multiple. The most typical site for primary strictures is the terminal ileum and the ileocecal valve.² Current medical therapy has shown limited efficacy in the management of fibrostenotic strictures and surgery remains the mainstay in these situations. When surgery is not feasible (patients with short gut, multiple prior

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Abbreviations used in this paper: AL, anastomotic leak; CAN, colitis-associated neoplasia; CD, Crohn's disease; DCE, dye-based chromoendoscopy; EBD, endoscopic balloon dilation; ESD, endoscopic submucosal dissection; EST, endoscopic stricturotomy; FCSEMS, fully covered self-expanding metal stent; FICE, flexible spectral imaging color enhancement; IBD, inflammatory bowel disease; ICA, ileocolonic anastomosis; UC, ulcerative colitis; VCE, virtual chromoendoscopy.



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Table 1. Do's and Don'ts in IBD Therapeutic Endoscopy

	Do's	Don'ts
Colitis-associated neoplasia	Surveillance yearly for primary sclerosing cholangitis Surveillance with ideal bowel prep Minimal to no inflammation; If inflammation is present, repeat colonoscopy within 1 y after optimization of therapy Enhanced imaging, high-definition DCE ideally but high-definition virtual chromoendoscopy if trained in it Describe lesions using the 5 Ss: size, shape, site (location in colon), surface, surrounding mucosa (active inflammation or not) Endoscopic resection when feasible Random biopsies for high-risk patients Follow tailored surveillance guidelines	Survey at standard intervals in inadequate prep or active inflammation Partially resect lesions Refer to colectomy if lesions are resectable Perform yearly surveillance on all patients
Anastomotic bleeding	Endoscopic clipping and a spray of hemostatic gel can be used for the treatment of anastomotic bleeding Endoscopic clipping can be attempted in acute anastomotic leaks or the internal opening of the fistula from chronic anastomotic leaks Endoscopic stent drainage can be used for the treatment of acute anastomotic leaks Endoscopic sinusotomy can be used for the treatment of chronic anastomotic leak-associated sinus	Topical injection of epinephrine or angioembolization should be avoided in the treatment of anastomotic bleeding Endoscopic clipping should be avoided in disease-associated fistula or chronic anastomotic leak-associated sinus
Endoscopic stricture management	Obtain a preprocedure computed tomography enterography/magnetic resonance enterography Discontinue antithrombotic and antiplatelet agents Perform graded dilatation Use wire-guided CRE balloons Biopsy the stricture postdilatation to rule out malignancy Follow-up endoscopy to assess the long-term response to endotherapy	Stop biological or advanced medical therapies during endotherapy for strictures Inject corticosteroid into the strictures after endotherapy Perform EBD if stricture is associated with a fistula or abscess Prophylactic antibiotics are not required prior to endotherapy Perform EBD if deep ulcerations are present secondary to inflammation, optimize inflammation management first.
Endoscopic fistula management	Obtain preprocedure imaging Optimize medical management for fistulas Choose short and simple fistulas for endotherapy Prioritize drainage over closure of fistula	Drain endoscopically complex fistulas Close the exit site of the fistula

CRE, controlled radial expansion; DCE, dye-based chromoendoscopy.

surgeries, rigid/fixed abdomen, overall poor medical condition), endoscopy is a valid and valuable approach.

Reviewing preprocedure cross-sectional imaging to better evaluate the length of the stricture, identify significant prestenotic dilatation, detect any associated fistulas, and ascertain the presence of significant inflammation is critical (Table 1 and Figure 1).³ Based on published literature, short strictures (length <4–5

cm), and patients with fewer than 3 risk factors (fistulizing disease, prestenotic dilatation >5 cm, high C-reactive protein, or prior anti-tumor necrosis factor exposure) are suitable for endoscopic therapy.⁴ Any number of strictures may be treated endoscopically if technically possible.⁵ Longer strictures (>4–5 cm), multiple strictures (>4), or those with significant risk factors (≥3) may require surgical intervention.^{3,5–7}

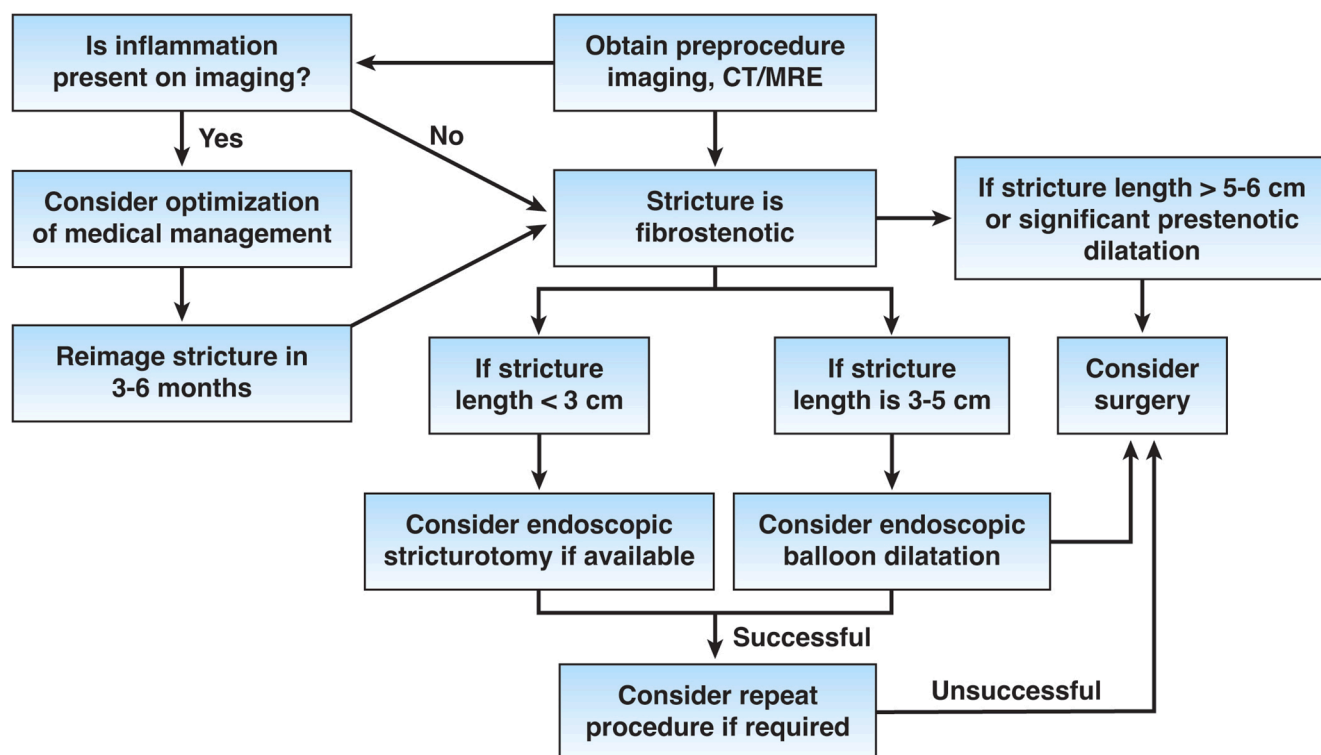


Figure 1. Flowchart to the approach and management of patients with intestinal stricture. After a successful EBD or EST, both can be repeated if successful, if unsuccessful then can be proceeded to surgery. CT, computed tomography; MRE, magnetic resonance enterography.

Endoscopic Balloon Dilation

Endoscopic balloon dilation (EBD) is one of the most commonly employed methods for treating strictures. Its ease of use, availability, and high technical, and clinical success rates (74%–100%) make it a first-line option (Table 2). Certain factors make EBD less likely to succeed, including longer stricture length (>4–5 cm), high body mass index, proximal small bowel strictures, significant prestenotic dilatation (>5 cm in the small bowel), and primary strictures.⁸ A stricture length of ≤5cm strongly predicts surgery-free survival, with the risk of surgery increasing by 8% for every 1 cm increase in stricture length. Symptomatic recurrence occurs in approximately 50% of patients, with two-thirds eventually requiring additional dilation or surgical intervention over a follow-up period ranging from 20 to 144 months.⁸

When performing EBD, retrograde dilation, defined as dilation in which the scope is able to pass the stricture to the proximal end and dilation performed when the scope is pulled back to the distal end of the stricture, is preferred whenever feasible.³ Using a guidewire is encouraged, particularly for impassable or angulated strictures. The use of fluoroscopy depends on the stricture location and the endoscopist's experience. A graded dilation method should be used, involving controlled radial expansion during inflation and partial or complete deflation between expansions (Figure 2G and H). The

target size for dilation is 18–20 mm, which may sometimes require more than 1 session.³ It is important to note that there is currently no convincing evidence linking balloon size to the risk of perforation. Therefore, caution is advised during EBD, with reassessment of the stricture after each dilation, as perforation may occur even with smaller balloon sizes. There is no benefit from intralesional steroids based on a recent consensus statement.⁷ EBD can be performed for strictures located anywhere in the gastrointestinal tract, including anastomotic locations. Ostomy site strictures can develop due to tissue ingrowth, stoma site retraction, or in patients with a continent ileostomy (Kock or "K" pouch), and EBD can be performed safely for ostomy-associated strictures. EBD should be avoided if there are associated deep ulcers or if a fistula is present, as it may worsen the fistula.³

Future advancements in EBD include drug-coated balloon and tools like endoluminal functional lumen imaging probe (Endo-Flip; Medtronic) to assess for stricture distensibility postdilatation.

Endoscopic Stricturotomy

Endoscopic stricturotomy (EST) is a technique in which an electro-incision knife is used to dissect and remove strictured tissue using electrocautery. EST can be performed in a radial, horizontal, or circumferential fashion.³ EST is a relatively new technique currently

Table 2. Notable Studies in the Interventional IBD

Author Year	Intervention	Study Type	Patients	Outcomes	Adverse events
Andujar X 2020 ²⁶	EBD	Retrospective cohort study	187	79.2% technical success	1.3%
Hirai F 2018 ²⁷	EBD	Prospective study	95	93.7% technical success	5%
Lan N 2018 ⁹	EST	Retrospective cohort study	185	100%	3.7%
Loras C 2022 ¹¹	Enteral Stenting	Prospective randomized trial	80	100% technical success	3%
Kochhar G 2018 ¹³	Endoscopic fistulotomy	Retrospective case series	29	100%	3.4%
TeGroen M 2025 ²⁸	Dysplasia detection	Randomized clinical trial	563	High-definition white light endoscopy with segmental re-inspection was non-inferior to high-definition chromoendoscopy for colorectal neoplasia detection in IBD patients	None
Maselli R 2025 ²⁹	ESD	Retrospective multicenter cohort study	91	With ESD or hybrid ESD, en bloc, R0, and curative resections were achieved in 95.8%, 85.4%, and 83.3%, respectively	12.5%

EBD, endoscopic balloon dilation; ESD, endoscopic submucosal dissection; EST, endoscopic stricturotomy.

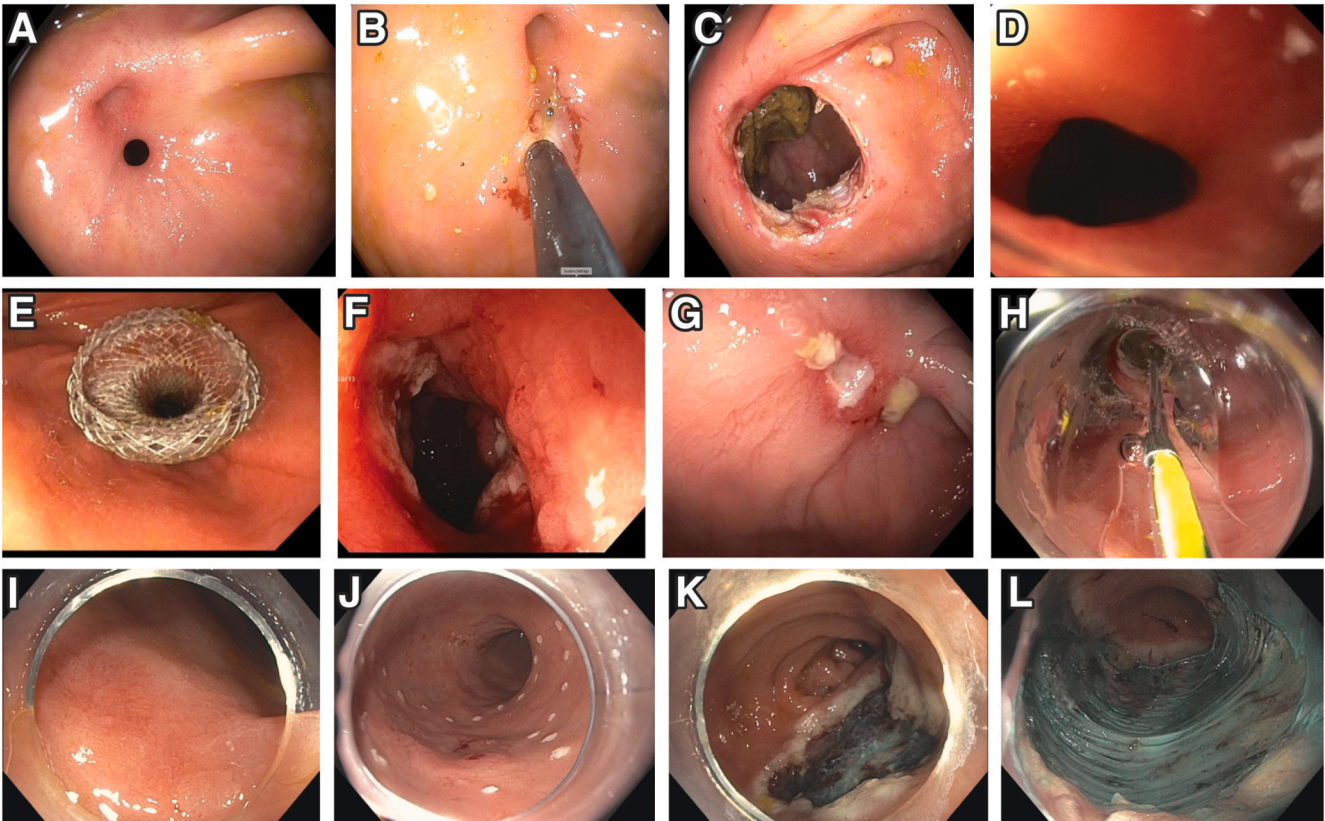


Figure 2. Representative images of before and after endoscopic treatment modalities: (A) cecum stricture, (B) treated with endoscopic stricturotomy, (C) postendoscopic stricturotomy, (D) pylorus stricture, (E) successful placement of enteral stent across pylorus, (F) postenteral stent removal, (G) stricture at terminal ileum and cecum, (H) endoscopic balloon dilatation of the stricture, (I) flat dysplasia in sigmoid colon, (K) Status post endoscopic thermal marking; (L and M) Status post endoscopic submucosal dissection.

being performed in few centers. In our experience, EST most effectively works for short fibrotic strictures (1–3 cm).

The technique for performing EST depends on the location and length of the stricture (Figure 2A–C). The length of the knife serves as a guide to regulate the incision depth, with incisions confined to the submucosa.³ EST can be performed as a stand-alone technique or as part of a hybrid technique combined with EBD. EST offers greater efficacy and a lower risk of perforation than EBD, although it carries a higher risk of delayed bleeding (5%–6%).⁹

A recent systematic review and meta-analysis, including 640 EST procedures performed on 287 IBD patients (169 with CD and 118 with ulcerative colitis), reported a technical success rate of 96.4% and a clinical success rate of 62%. The mean stricture length was 1.68 ± 0.84 cm. During a mean follow-up period of 1.0 ± 1.1 years, 16.4% of patients required surgery, while 44.2% required additional endoscopic treatments, such as repeat EST or EBD.¹⁰ EST may be considered based on the endoscopist's experience for all areas in the gastrointestinal tract including anal canal. Anal canal strictures are not ideally suited for surgical management, and EBD is a viable option for these strictures. However, in the authors' experience, EST remains more effective than EBD for such strictures. Another option for anal canal strictures is self-dilatation using graded metallic dilators.

Enteral Stenting

Enteral stenting (ES) has been an area of interest for managing strictures in patients with IBD. The initial experience was not encouraging, recent advances in stent designs and deployment techniques have renewed the interest in this approach. Currently, available stent options include fully covered self-expanding metal stents (FCSEMS), uncovered SEMS, partially covered SEMS, biodegradable stents, and lumen-apposing metal stents. Uncovered stents should almost never be used for benign diseases, and lumen-apposing metal stents are not currently approved for luminal use by the Food and Drug Administration. In the recent randomized controlled ProtDilat trial, ES was compared with EBD and the results showed that EBD was more effective than FCSEMS, as the need for repeat procedures was lower in the EBD group.¹¹ The study was well-conducted, it is essential to note that the FCSEMS used in the trial were not designed for small bowel or ileocolonic anastomoses; they were originally intended for the esophagus and were repurposed for small bowel use. This critical detail must be considered when evaluating trials comparing FCSEMS to EBD. In a recent meta-analysis, 9 studies involving 163 patients who underwent stenting were included, and results showed a pooled clinical success rate at

60.9% (95% confidence interval, 51.6%–69.5; $I^2 = 13\%$) and a pooled technical success rate at 93% (95% confidence interval, 87.3%–96.3%; $I^2 = 0\%$). The pooled rates of overall adverse events, proximal stent migration, and perforation were 15.7%, 6.4%, and 2.7%, respectively.¹² The pooled rate of spontaneous stent migration was 43.9% (95% confidence interval, 11.4%–82.7%; $I^2 = 88\%$). Despite its advantages, ES has several challenges, the most significant being the risk of stent migration. Currently, ES use is highly selective, not routinely performed and societies such as ASGE advise against it.⁷ It is reserved for refractory cases, strictures that do not respond to EBD or EST in patients who are not surgical candidates. With advancements in ES design, delivery mechanisms, and improved efficacy, ES may play a more significant role in managing strictures in patients with IBD in the future (Figure 2D–F).

Fistula and Abscess

Historically, most patients with structural phenotypes of IBD, including fistulizing CD, get treated with surgical intervention. However, patients with certain phenotypes of fistulizing CD may be treated with an endoscopic approach. For example, endoscopic fistulotomy may be performed in patients with a short (<3 cm) ileocecal fistula from long-standing ileocecal valve stenosis (Table 2).¹³ Patients with perianal abscesses with or without associated fistula can be treated with incision and drainage, or endoscopic wire-guided placement of setons in the endoscopy suite. Endoscopic closure of disease-associated fistulas in CD is usually ineffective, although the endoscopic approach can be attempted in anastomotic leak-associated fistulas. Newer modalities like endoscopic ultrasound guided drainage of abscess using lumen-apposing metal stents have also been showed to be feasible in draining of abscess in patients with IBD.

Colitis-Associated Neoplasia

A significant long-term complication of IBD is the increased risk of colorectal dysplasia and adenocarcinoma (colorectal cancer).¹⁴ Recent incidence rates of colitis-associated neoplasia (CAN) declining, with rates of 1%, 2%, and 5% risk at 10, 20, and >20 years, respectively, after the onset of disease, respectively.¹⁵ CAN is often flat and hard to detect.¹⁶ Endoscopic options to enhance dysplasia detection in IBD include dye-based chromoendoscopy (DCE) and virtual chromoendoscopy. Multiple international guidelines recommend chromoendoscopy as the optimal choice for surveillance.^{17–19} A network meta-analysis found DCE detects 1.42 times (95% confidence interval, 1.02–1.98) more dysplastic lesions compared with high-definition white light endoscopy.²⁰ Previous meta-analyses showed no

difference between high-definition DCE and virtual chromoendoscopy, including narrow-band imaging. The HELIOS randomized trial included 563 patients and showed that high-definition white light endoscopy with segmental re-inspection is noninferior to high-definition DCE for detecting CAN in IBD with a significantly shorter withdrawal time.²¹ DCE is ineffective with inadequate bowel preparation or active inflammation, requires training, and prolongs withdrawal time, limiting its widespread uptake in clinical practice. Random biopsies during surveillance colonoscopy are still advised for high-risk patients, including those with primary sclerosing cholangitis, prior dysplasia, and active inflammation.¹⁷⁻¹⁹

Use standardized descriptive terminology in reporting while describing dysplastic lesions, by utilizing the modified Paris classification. The size, shape, site, surface pattern, and surrounding mucosal disease activity should be reported.¹⁹ Dysplasia identified on nontargeted/random biopsies should be labeled as “invisible” dysplasia. Description of the surrounding mucosa allows better planning for endoscopic resection, as poorly defined lesion borders, active inflammation, and ulceration within the lesion are all hindrances to effective removal.

Per guidances, CAN should be resected endoscopically when complete resection is feasible.^{18,19} High-risk lesions (mucosal ulceration, radiating folds, inability to lift, ill-defined margins, and invasive cancer on histology) should be referred for surgical resection.¹⁹ Also, multifocal unresectable or invisible dysplasia should be treated with surgical resection.¹⁹ If there is active background inflammation, medical therapy can be optimized, and the patient brought back in 3–6 months for a repeat colonoscopy with curative intent. Standard polypectomy techniques, including cold snare polypectomy, are adequate for removing small (<10 mm) well-defined lesions. Endoscopic mucosa resection and endoscopic submucosal dissection (ESD) are effective for larger lesions. In a meta-analysis of over 600 lesions with a mean size of 23 mm removed by endoscopic mucosa resection or ESD, complete resection was achieved in 97.9%.²² Local recurrence occurred in 4.9% of patients and metachronous lesions in 7.4%, emphasizing the need for continued high-quality surveillance. (Figure 2I, K–M). ESD can be technically challenging in patients with IBD due to submucosal fibrosis and fat expansion, rendering dissection and plane recognition much harder. Endoscopic full-thickness resection is now indicated in non-IBD cases for scarred, residual, recurrent, or nonlifting lesions.²³ There have been case reports of endoscopic full-thickness resection in CAN with good efficacy, but long-term outcome data are lacking. The evidence suggests the detection rate of dysplasia in biopsies surrounding dysplastic lesions is low, hence routine biopsies are not routinely required unless there are specific concerns about the resection completeness.¹⁹ Once IBD CAN lesions are resected, follow-up surveillance intervals must be tailored to the findings per multiple national and international guidances.^{18,19}

As IBD therapeutic options expand, we can expect an increasing number of patients to avoid colectomy, albeit with an increase in CAN over time. Gastroenterologists must be proficient in detecting and managing colitis-associated dysplasia to reduce overall mortality in these patients.

IBD Surgery–Associated Anastomotic Complications

At least half of patients with CD and around 10%–15% of patients with UC will eventually require surgery for the management of medically refractory disease, structural or neoplastic complications.²⁴ Postsurgical complications are common, mainly bleeding and leaks at the ileocolonic anastomosis (ICA), ileorectal anastomosis, colorectal anastomosis, and pouch-anal anastomoses.²⁴

Anastomotic Leaks

Anastomotic leaks (ALs) are classified into acute (within 30 days) and chronic. The usual locations of anastomotic leaks are ICA per se and transverse staple line of the side-to-side ICA in CD, the tip of the J pouch. Small ALs can be self-limited and can be managed with conservative therapy, or with diverting ileostomy. Large (>3 cm) acute AL-associated abscesses can be drained subcutaneously (ie external drainage) via interventional radiology or endoscopically with the placement of drainage endosponge or drainage stents (ie, internal drainage). Post-abscess drainage endoscopic closure with through-the-scope or over-the-scope clips may be attempted.

Chronic AL-associated sinuses or fistulas may also be treated with endoscopic sinusotomy or fistulotomy.²⁵ Endoscopic closure of chronic ALs with over-the-scope clips from the transverse staple line at ICA in CD or at the tip of the J in ileal pouch-anal anastomosis may be attempted.

Future Perspectives

The field of interventional IBD has experienced a tremendous growth and expect more gastroenterologists, advanced endoscopists, and IBD specialists to perform endoscopic therapy for IBD and IBD surgery–associated complications. We foresee increased applications of endoscopic technology (such as endoscopic ultrasound, intraoperative enteroscopy, image-enhanced endoscopy, and artificial intelligence), endoscopic techniques (such as existing and new modalities of treatment of strictures, prevention and damage controls of endoscopic complications) in IBD.

References

1. Hashash JG, Thompson CC, Wang AY. AGA rapid clinical practice update on the management of patients taking GLP-1

- receptor agonists prior to endoscopy: communication. *Clin Gastroenterol Hepatol* 2024;22:705–707.
2. Chan WPW, Mourad F, Leong RW. Crohn's disease associated strictures. *J Gastroenterol Hepatol* 2018;33:998–1008.
 3. Shen B, Kochhar G, Navaneethan U, et al. Practical guidelines on endoscopic treatment for Crohn's disease strictures: a consensus statement from the Global Interventional Inflammatory Bowel Disease Group. *Lancet Gastroenterol Hepatol* 2020;5:393–405.
 4. Bossuyt P, Debeuckelaere C, Ferrante M, et al. Risk stratification for surgery in stricturing ileal Crohn's disease: the BACARDI risk model. *J Crohns Colitis* 2018;12:32–38.
 5. Yamamoto H, Yano T, Araki A, et al. Guidelines for endoscopic balloon dilation in treating Crohn's disease-associated small intestinal strictures (supplement to the Clinical Practice Guidelines for Enteroscopy). *Dig Endosc* 2022;34:1278–1296.
 6. Adamina M, Minozzi S, Warusavitarne J, et al. ECCO guidelines on therapeutics in Crohn's disease: surgical treatment. *J Crohns Colitis* 2024;18:1556–1582.
 7. Shen B, Abreu MT, Cohen ER, et al. Endoscopic diagnosis and management of adult inflammatory bowel disease: a consensus document from the American Society for Gastrointestinal Endoscopy IBD Endoscopy Consensus Panel. *Gastrointest Endosc* 2025;101:295–314.
 8. Bettenworth D, Bokemeyer A, Kou L, et al. Systematic review with meta-analysis: efficacy of balloon-assisted enteroscopy for dilation of small bowel Crohn's disease strictures. *Aliment Pharmacol Ther* 2020;52:1104–1116.
 9. Lan N, Shen B. Endoscopic stricturotomy versus balloon dilation in the treatment of anastomotic strictures in Crohn's disease. *Inflamm Bowel Dis* 2018;24:897–907.
 10. Jaber F, Numan L, Ayyad M, et al. Efficacy and safety of endoscopic stricturotomy in inflammatory bowel disease-related strictures: a systematic review and meta-analysis. *Dig Dis Sci* 2024;69:4152–4166.
 11. Loras C, Andújar X, Gornals JB, et al. Self-expandable metal stents versus endoscopic balloon dilation for the treatment of strictures in Crohn's disease (ProtDilat study): an open-label, multicentre, randomised trial. *Lancet Gastroenterol Hepatol* 2022;7:332–341.
 12. Chandan S, Dhindsa BS, Khan SR, et al. Endoscopic stenting in Crohn's disease-related strictures: a systematic review and meta-analysis of outcomes. *Inflamm Bowel Dis* 2023;29:1145–1152.
 13. Kochhar G, Shen B. Endoscopic fistulotomy in inflammatory bowel disease (with video). *Gastrointest Endosc* 2018;88:87–94.
 14. Axelrad JE, Hashash JG, Itzkowitz SH. AGA clinical practice update on management of inflammatory bowel disease in patients with malignancy: commentary. *Clin Gastroenterol Hepatol* 2024;22:1365–1372.
 15. Castaño-Milla C, Chaparro M, Gisbert JP. Systematic review with meta-analysis: The declining risk of colorectal cancer in ulcerative colitis. *Aliment Pharmacol Ther* 2014;39:645–659.
 16. Bak MTJ, Albeniz E, East JE, et al. Endoscopic management of patients with high-risk colorectal colitis-associated neoplasia: a Delphi study. *Gastrointest Endosc* 2023;97:767–779e6.
 17. Bisschops R, East JE, Hassan C, et al. Advanced imaging for detection and differentiation of colorectal neoplasia: European Society of Gastrointestinal Endoscopy (ESGE) guideline - update 2019. *Endoscopy* 2019;51:1155–1179.
 18. Gordon H, Biancone L, Fiorino G, et al. ECCO guidelines on inflammatory bowel disease and malignancies. *J Crohns Colitis* 2023;17:827–854.
 19. Murthy SK, Feuerstein JD, Nguyen GC, et al. AGA clinical practice update on endoscopic surveillance and management of colorectal dysplasia in inflammatory bowel diseases: expert review. *Gastroenterology* 2021;161:1043–1051.e4.
 20. Sinopoulou V, Nigam GB, Gordon M, et al. Comparative efficacy and safety of endoscopic modalities for colorectal cancer screening in inflammatory bowel disease: a systematic review and network meta-analysis. *Clin Gastroenterol Hepatol* 2025;23:2128–2143.
 21. te Groen M, Derks M, den Broeder N, et al. Quality of surveillance impacts the colitis-associated advanced neoplasia risk: a multicenter case-control study. *Clin Gastroenterol Hepatol* 2024;22:357–367.e5.
 22. Mohapatra S, Sankaramangalam K, Lopimpisuth C, et al. Advanced endoscopic resection for colorectal dysplasia in inflammatory bowel disease: a meta-analysis. *Endosc Int Open* 2022;10:E593–E601.
 23. Barbaro F, Papparella LG, Chiappetta MF, et al. Endoscopic full-thickness resection vs. endoscopic submucosal dissection of residual/recurrent colonic lesions on scars: a retrospective Italian and Japanese comparative study. *Eur J Gastroenterol Hepatol* 2024;36:162–167.
 24. Shen B, Kochhar GS, Navaneethan U, et al. Endoscopic evaluation of surgically altered bowel in inflammatory bowel disease: a consensus guideline from the Global Interventional Inflammatory Bowel Disease Group. *Lancet Gastroenterol Hepatol* 2021;6:482–497.
 25. Lan N, Hull TL, Shen B. Endoscopic sinusotomy versus redo surgery for the treatment of chronic pouch anastomotic sinus in ulcerative colitis patients. *Gastrointest Endosc* 2019;89:144–156.
 26. Andújar X, Loras C, González B, et al. Efficacy and safety of endoscopic balloon dilation in inflammatory bowel disease: results of the large multicenter study of the ENEIDA registry. *Surg Endosc* 2020;34:1112–1122.
 27. Hirai F, Andoh A, Ueno F, et al. Efficacy of endoscopic balloon dilation for small bowel strictures in patients with Crohn's disease: a nationwide, multi-centre, open-label, prospective cohort study. *J Crohns Colitis* 2018;12:394–401.
 28. Te Groen M, Wijnands AM, den Broeder N, et al. Surveillance in inflammatory bowel disease: white light endoscopy with segmental re-inspection versus dye-based chromoendoscopy - a multi-arm randomised controlled trial (HELIOS). *Gut* 2025;74:547–556.
 29. Maselli R, de Sire R, Barbaro F, et al. Outcomes of endoscopic submucosal dissection for high-risk colorectal colitis-associated neoplasia in inflammatory bowel disease. *Endoscopy* 2025;57:658–666.

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Conflicts of interest

These authors disclose the following: Gursimran S. Kochhar has served as a speaker for Eli Lilly; has served on the advisor board for Eli Lilly and GIE Medical; and has served as a consultant for Takeda Pharma, Boston Scientific, Olympus Endoscopy, Exact Sciences, Pentax Endoscopy; and owns stock options in Digbi Health. Nayantra Coelho-Prabhu has served as a consultant for Boston Scientific and Iterative Health. Bo Shen has served as a consultant for Janssen; and received research/education grants from AbbVie, GIE Medical, Janssen, and Takeda. The remaining author discloses no conflicts.